

Week 7 — Diagnosis by Inquiry & Palpation

EAM310 · EAST ASIAN MEDICINE DIAGNOSIS I · VUIM SUMMER 2026

△ Quiz 5 — August 28, 2026

Class: Friday August 28, 2026 · 9AM–1PM

Prof. Daniel Schrier, D.O.M., L.Ac.

Jonathan Centeno, D.AcHM Candidate

REQUIRED READINGS — DUE BEFORE AUGUST 28

FCM CHAPTER 24 — INQUIRY — MACIOCIA

- Diagnosis by Interrogation — full chapter
- General vs Specific interrogation
- The 16 Questions — full system, all categories
- Terminology: Chinese → Western patient language

FCM CHAPTER 25 — PALPATION — MACIOCIA

- Palpating skin: temperature, moisture, texture
- Palpating limbs, chest, abdomen
- Diagnostic points: Back-Shu, Front-Mu, Lower He-Sea
- CAM Ch.9 — Inquiry (問診) & Palpation (切診)

WEEK 7 LEARNING OBJECTIVES

- Distinguish General vs Specific interrogation
- Interpret thirst, stools, urine, sweat findings
- Interpret skin temperature at graded pressure depths
- Interpret chest, abdomen, and point-palpation findings
- Recite the 16 Questions in Maciocia's revised order
- Apply Maciocia's 8-step full intake procedure
- Differentiate Full-Heat vs Empty-Heat by dorsum/palm
- Identify Back-Shu, Front-Mu, Lower He-Sea point locations

ASSESSMENT CALENDAR

Date	Event	Material Covered
Aug 21	Week 6 Class	Diagnosis by Observation
Aug 28	△ Quiz 5	Diagnosis by Inquiry & Palpation — 16 Questions, skin/chest/abdomen/points
Sep 4	△ Quiz 6	Week 8 material — Hearing & Smelling
Sep 11	FINAL + Journal 26–50	Weeks 6–9 comprehensive

IN-CLASS PRACTICE — WHAT TO EXPECT

INTERROGATION & FULL-BODY PALPATION LAB

Week 7 combines partner interview practice with hands-on palpation. Come ready to:

- Conduct a mock intake using Maciocia's 8-step procedure on a partner
- Compare dorsum vs palm temperature (Full-Heat vs Empty-Heat)
- Palpate the apical beat (Xu Li) and describe strength/quality
- Locate and palpate at least 2 Back-Shu and 2 Front-Mu points, noting tenderness
- Document findings in your Pulse/Tongue Journal

TWO ASPECTS OF INTERROGATION 問診	
General	Talk between doctor and patient about how the problem arose, living/working conditions, emotional and family environment. Aim: find the cause , not the pattern.
Specific	Aims at identifying the prevailing pattern(s) of disharmony — by Internal Organs, by channels, by Four Levels, etc. Aim: find the pattern .

Maciocia's own practice: "I personally concentrate first on the clinical manifestations to identify the patterns involved. It is only after I have come to a conclusion about the patterns involved that I ask general questions about lifestyle, work, emotions... to try and find the cause." Pattern first, cause second — don't blur the two.

"SYMPTOMS" IN CHINESE MEDICINE — BROADER THAN THE WEST	
Western medicine: symptoms = subjective/objective manifestations of a disease process. Chinese medicine: symptoms are ANY manifestation that forms part of the whole clinical picture — many are unrelated to a definable "disease."	
"Non-Western" Symptom	What It Points To
Absence of thirst	Confirms a Cold condition
Incapacity to make decisions	Deficiency of the Gall Bladder
Dislike of speaking	Deficiency of the Lungs
Propensity to outbursts of anger	Rising Liver-Yang or Liver-Fire
Desire to lie down	Weakness of the Spleen
Dull appearance of the eyes	Disturbance of the Mind, emotional problems
Deep midline crack on the tongue	Propensity to deep emotional problems

TERMINOLOGY PROBLEM — CHINESE → WESTERN PATIENT LANGUAGE	
Chinese patients spontaneously describe a "distending pain." An English-speaking Western patient instead says "bloated" or "bursting." Learn to translate — the pattern logic doesn't change, but the vocabulary does.	

MACIOCIA'S 8-STEP FULL INTAKE PROCEDURE	
1	Let the patient speak freely about the presenting problem
2	Ask specific questions to determine the pattern, establishing onset
3	Ask questions to exclude or confirm other patterns
4	Broadly follow the 16 Questions
5	Ask about past diseases and operations
6	Look at the tongue, feel the pulse
7	Ask about family illnesses
8	Inquire about emotional/family/work life to find the cause

Golden rule: "What matters is the general PICTURE formed by the symptoms... it is only for didactic purposes that we list symptoms in isolation." Never diagnose off one isolated answer — always integrate the full clinical picture.

THE CLASSICAL TEN-QUESTION SONG 十問歌 (ZHANG JINGYUE)			
#	Classical Question	#	Classical Question
1	Cold and heat 寒热	6	Chest and abdomen 胸腹
2	Sweat 汗	7	Deafness/hearing 聾
3	Head and body 头身	8	Thirst 渴
4	Stool and urine 二便	9	Old disease 旧病
5	Food and drink 饮食	10	Cause 因

10 → 16, what actually changed: Maciocia didn't discard Zhang Jingyue's classical 10 — he split compound categories apart (Chest/Abdomen, Head/Body become distinct), promoted implicit categories to full standalone items (Energy, Emotional, Sexual, Women's, Children's), then reordered so Pain leads and Chills/Fever confirms at the end. Same tradition, modernized packaging.

Why the order changed: Maciocia moved Pain to #1 (Western patients' most common complaint) and Chills/Fever to #12, only asked at the end to CONFIRM the Hot/Cold nature of an already-identified pattern. The classical Qing-dynasty 10 questions assumed acute exterior febrile disease (Shang Han Lun bias).

THE 16 QUESTIONS — MACIOCIA'S ORDER

#	Area	#	Area
1	Pain	9	Sleep
2	Food and taste	10	Sweating
3	Stools and urine	11	Ears and eyes
4	Thirst and drink	12	Cold, heat, fever
5	Energy levels	13	Emotional symptoms
6	Head, face, body	14	Sexual symptoms
7	Chest and abdomen	15	Women's symptoms
8	Limbs	16	Children's symptoms

Q1 — MASTER PAIN TABLE — FULL VS EMPTY, COLD VS HEAT

Factor	Empty	Full	Cold	Heat
Pressure	Alleviated	Aggravated	—	—
Type	Dull, lingering	Sharp	Cramping	Burning
Temperature	—	—	Better w/ heat	Better w/ cold
Onset	Slow, gradual	Sudden	—	—
Rest/movement	Better w/ rest	Better w/ movement	Better w/ movement	Worse w/ movement

Cause	Character	Location
Qi stagnation	Distension > pain, no fixed location	Epigastrium, abdomen
Blood stasis	Boring/stabbing, FIXED	Head, chest, epigastrium, Uterus
Cold	Cramping, better with heat	Epigastrium, Uterus, joints
Damp-Heat	Burning + fullness/heaviness	Forehead, epigastrium, Bladder, joints
External Wind	—	Occiput

Q2 — FOOD & TASTE

Finding	Pattern
Relieved by eating	Empty pattern
Aggravated by eating	Full pattern
Lack of appetite	Spleen-Qi deficiency
Excessive hunger	Stomach-Heat
Fullness after eating	Retention of food or Dampness
Distension	Qi stagnation
Sharp pain	Blood stasis
Heaviness	Dampness
Preference for hot food	Cold pattern
Preference for cold food	Heat pattern

Taste	Pattern
Bitter	Full-Heat, Liver or Heart (constant = LV-Fire; only after sleepless night = HT-Fire)
Sweet	Spleen deficiency or Damp-Heat
Sour	Retention of food, or Liver invading Stomach
Salty	Kidney-Yin deficiency
Pungent	Lung-Heat
Lack of taste sensation	Spleen and Stomach deficiency

Q3 — STOOLS & URINE QUICK REFERENCE

Maciocia's 16 Questions combine Stools and Urine as ONE question (#3) — covered here as two sub-tables.

Stools	Pattern
Constipation, worse after BM	Empty pattern

Stools	Pattern
Improves after BM	Full pattern
Constipation + thirst + yellow coat	Heat in Stomach/Intestines
Constipation, elderly/postpartum	Blood deficiency
Goat-stool constipation	Liver-Qi stagnation + Intestinal Heat
Chronic loose stools/diarrhoea	Spleen and/or Kidney-Yang deficiency
Diarrhoea every early morning	Kidney-Yang def ("cock-crow diarrhoea")
Diarrhoea + mucus	Dampness in Intestines
Diarrhoea + mucus + blood	Damp-Heat in Intestines
Black or very dark stools	Stasis of Blood
Borborygmi + loose stools	Spleen deficiency
Borborygmi + distension, no loose stools	Liver-Qi stagnation
Foul-smelling stools	Heat
Absence of smell	Cold

Urine	Pattern
Enuresis/incontinence	Kidney deficiency
Frequent + copious	Kidney-Yang deficiency
Frequent + scanty	Qi deficiency
Scanty, dark	Kidney-Yin deficiency
Pain before / during / after	Qi stag / Damp-Heat BL / Qi def
Turbid, cloudy	Dampness in Bladder

Q4 — THIRST QUICK REFERENCE	
Large cold drinks	Full-Heat (any organ)
Small sips	Yin deficiency (ST/KD)
No thirst	Cold (ST/SP)
Thirst, no desire to drink	Damp-Heat
Desire cold liquids	Heat pattern
Desire warm liquids	Cold pattern

Q5 — ENERGY LEVELS (TIREDNESS)
Not one of the classical 10 — Maciocia added it because tiredness is one of the most common Western complaints (~12% of his patients sought treatment for tiredness alone). Ask early, as soon as a Deficiency pattern is suspected. Remember: tiredness is not always Deficiency — it can be Full (Dampness, Phlegm, Qi stagnation), especially with a Slippery or Wiry pulse.

Chronic Tiredness + ...	Pattern
Desire to lie down, poor appetite, loose stools	Spleen-Qi or Spleen-Yang deficiency
Weak voice, catches colds easily	Lung-Qi or Lung-Yang deficiency
Backache, feeling cold, frequent urination	Kidney-Yang deficiency
Dizziness, blurred vision, scanty periods	Liver-Blood deficiency
Anxiety, dry mouth at night, no tongue coating	Kidney-Yin deficiency
Feeling of heaviness	Dampness
Heaviness + muzziness + dizziness	Phlegm
Abdominal distension, irritability, Wiry pulse	Liver-Qi stagnation
Alternating hot/cold, unilateral tongue coat, Wiry pulse	Lesser Yang (Shaoyang) pattern

Q6 — HEAD, FACE & BODY: HEADACHE	
Onset/Timing	Pattern
Recent, short duration	Exterior Wind-Cold
Gradual, in attacks	Interior type
Daytime	Qi or Yang deficiency
Evening	Blood or Yin deficiency
Night-time	Blood stasis
Location	Channel / Cause
Nape of neck	Greater Yang (ext. Wind-Cold or int. KD def)
Forehead	Bright Yang (ST-Heat or Blood def)

Location	Channel / Cause
Temples/sides	Lesser Yang (ext. Wind or LV/GB Fire)
Vertex	Terminal Yin (usu. LV-Blood def)
Whole head	Exterior Wind-Cold

Character / Condition	Pattern
Heavy feeling	Dampness or Phlegm
Pain "inside," "hurting the brain"	Kidney deficiency
Distending, throbbing	Liver-Yang rising
Boring, like a nail	Blood stasis
Aggravated by fatigue, better with rest	Qi deficiency
Aggravated by emotional tension	Liver-Yang rising
Aggravated by sexual activity	Kidney deficiency

Dizziness quick reference: Severe giddiness/loss of balance = internal Wind · With throbbing headache = Liver-Yang rising · With heaviness/muzziness = Phlegm obstructing.

Q7 — CHEST & ABDOMEN

Stabbing chest pain	Stasis of Blood in the Heart
Chest pain + cough + yellow sputum	Lung-Heat
Oppression of the chest	Phlegm in Lungs, or severe LV-Qi stagnation
Hypochondrium distension	Liver-Qi stagnation
Hypochondrium stabbing pain	Liver-Blood stasis
Epigastric pain, dull, better w/ eating	Empty type (ST-Qi def or Empty-Cold)
Epigastric pain, severe, worse w/ eating	Full type (food retention, ST-Heat, Damp-Heat, Blood stasis, LV invading ST)
Epigastric fullness	Dampness, or slight SP-Qi deficiency
Epigastric distension	Liver-Qi stagnation
Lower abdominal pain	Interior Cold, LV-Qi/Blood stagnation, Damp-Heat, or Blood stasis in Intestines/Uterus
Abdominal pain relieved by BM	Full nature
Abdominal pain worse after BM	Empty nature
Hypogastric pain	Damp-Heat in Bladder, or LV-Fire infusing down

Palpation cross-check: A mass that MOVES under the fingers = Qi stagnation. A FIXED, hard mass = Blood stasis. Tenderness at Ren-17 (Shanzhong) on light pressure = classic Heart-Blood stasis sign.

Q8 — LIMBS & BODY

Only routinely asked about when the patient presents with a specific limb complaint — **except** always ask about numbness with Blood deficiency, and heaviness with Dampness.

Numbness, both sides (arms+legs or hands+feet)	Blood deficiency
Numbness, ONE side only (fingers/elbow/arm)	⚠ Internal Wind + Phlegm — possible impending Wind-stroke
Joint pain, wandering	Wind
Joint pain, fixed, very painful	Cold
Joint pain, fixed + swelling + numbness	Dampness
Joint swelling + redness	Damp-Heat
Severe stabbing pain + rigidity	Blood stasis
Backache, continuous, dull, better w/ rest	Kidney deficiency
Backache, recent onset, severe, stiff	Sprain causing Blood stasis
Backache, worse in cold/damp weather	Exterior Cold-Damp invading back channels
Backache, boring, can't turn waist	Blood stasis

Q9 — SLEEP & Q LETHARGY QUICK REFERENCE

Sleepy after eating	Spleen-Qi deficiency
General lethargy + heaviness	Retention of Dampness
Lethargy + heaviness + dizziness	Phlegm
Extreme lethargy + cold feeling	Kidney-Yang deficiency
Insomnia, dream-disturbed	Heart Blood deficiency
Difficulty falling asleep, restless	Heart Yin deficiency / Empty-Heat
Lethargic stupor + rattling throat + Slippery pulse	Phlegm blurring the Mind

Q10 — SWEATING: FOUR FACTORS TO EVALUATE

Factor	Finding	Meaning
AREA OF BODY		
Area	Only on head	Heat in the Stomach, or Damp-Heat
	Oily sweat on forehead	Collapse of Yang
	Only arms and legs	Stomach and Spleen deficiency
	Only on hands	Lung-Qi or Heart-Qi deficiency
	Whole body	Lung-Qi deficiency
	Palms, soles, chest ("five-palm sweat")	Yin deficiency
TIME OF DAY		
Timing	Daytime	Yang deficiency
	Night-time	Yin deficiency (occasionally Damp-Heat)
CONDITION OF ILLNESS		
Severity	Profuse cold sweat during severe illness	Collapse of Yang
	Oily sweat on forehead "like pearls," not flowing	Collapse of Yang — imminent danger
QUALITY OF SWEAT		
Quality	Oily	Severe Yang deficiency
	Yellow	Damp-Heat

Q11 — EARS & EYES

Tinnitus, sudden/loud	Full (LV-Fire, LV-Yang rising)
Tinnitus, gradual/faint	Empty (KD-Essence def)
Blurred vision	Liver-Blood deficiency
Dry eyes	Liver-Blood or Yin deficiency
Floaters	Liver-Blood or KD-Essence def

Q12 — COLD/HEAT/FEVER (CONFIRMS, NOT LEADS)

Aversion to cold, no sweat	Exterior Full (Wind-Cold)
Aversion to wind, some sweat	Exterior Empty (Wind, Upright Qi def)
Alternating chills & fever	Shaoyang / half-ext-half-int
Tidal fever (afternoon)	Yin deficiency / Empty-Heat
Aversion to cold, desires warmth	Interior Cold (Yang def)

Clinical logic: Q12 is placed near the END of the 16 Questions deliberately — by this point you likely already suspect Hot or Cold from pain, thirst, stools, and sweat findings. Cold/Heat/Fever questions *confirm* that suspicion; they rarely lead the diagnosis in modern Western practice the way they did in Shang Han Lun-era febrile disease.

Q13 — EMOTIONAL SYMPTOMS	
Depression, sighing	Liver-Qi stagnation
Irritability, anger outbursts	LV-Yang rising / LV-Fire
Anxiety, worry	Heart/Spleen involvement
Fear, timidity	Kidney involvement
Excessive grief	Lung involvement

Q14 — SEXUAL SYMPTOMS	
Ask gently, following the conversation — often skipped by modern Chinese doctors due to cultural prudery, but Maciocia insists it always be included.	
Low libido	KD-Yang or Essence deficiency
Premature ejaculation	KD-Qi not consolidating, or Heat
Intercourse during/soon after period	Qi/Blood stasis in Uterus (per Fu Qing-Zhu)

Q15 — WOMEN'S SYMPTOMS: MENSTRUATION		
Parameter	Finding	Pattern
CYCLE		
Timing	Always early	Blood-Heat or Qi deficiency
	Always late	Blood deficiency, stagnation, or Cold
	Irregular (early & late)	LV-Qi/Blood stagnation, or Spleen def
AMOUNT & COLOUR		
Amount/Colour	Heavy loss	Blood-Heat or Qi deficiency
	Scanty	Blood deficiency, stagnation, or Cold
	Dark-red / bright-red	Blood-Heat
	Purple/blackish with clots	Blood stasis or Cold
PAIN TIMING		
Pain	Before periods	Stagnation of Qi or Blood
	During periods	Blood-Heat or stagnation of Cold
	After periods	Blood deficiency
Leucorrhoea		
Colour	White	Cold, SP/KD-Yang def, or ext. Cold-Damp
	Yellow	Damp-Heat in Lower Burner
	Greenish	Damp-Heat in Liver channel

Q16 — CHILDREN'S SYMPTOMS (ASK CAREGIVER + DIRECT OBSERVATION)	
Appetite, energy, sleep	General Qi/constitution baseline
Behavior/mood changes	Screen for Liver/Heart disharmony early
Digestive symptoms	Spleen system especially vulnerable in children
Respiratory symptoms, earache	Lung/Kidney — children's Wei Qi still developing

SKIN TEMPERATURE — COLD PATTERNS

General cold feeling	Cold pattern
Four limbs cold	Spleen- and/or Stomach-Yang deficiency
Loins, lower back, or feet cold	Kidney-Yang deficiency
Lower abdomen cold	Spleen-Yang deficiency

SKIN TEMPERATURE — HEAT PATTERNS (PRESSURE-DEPENDENT DIAGNOSIS)

Skin actually feels hot to touch	Damp-Heat
Hot on FIRST touch, fades with sustained pressure	Exterior Wind-Heat — pathogen still on Exterior
Hot on MEDIUM pressure, NOT on heavy pressure	Interior Heat in Middle Burner or Heart
Hot on HEAVY pressure, nearly to the bone	Empty-Heat from Yin deficiency

Key diagnostic principle: The DEPTH at which heat is felt tells you the depth of the pathology. Superficial heat that fades = exterior. Heat that only appears deep = the deepest Yin-deficiency Empty-Heat.

SKIN MOISTURE & TEXTURE

Moist skin	Invasion of exterior Wind (Cold or, more usually, Heat)
Moist, NO exterior symptoms	Spontaneous sweating from Lung-Qi deficiency
Dry skin	Blood deficiency or Lung-Yin deficiency
Rough skin	Painful Obstruction Syndrome from Wind
Scaly and dry skin	Exhaustion of Body Fluids, or severe Liver-Blood deficiency/dryness
Swollen, PITTING on pressure	"Water swelling" — Spleen- and/or Kidney-Yang deficiency
Swollen, NOT pitting (no pit left)	"Qi swelling" — Dampness or Qi stagnation

COLD HANDS & FEET — FULL DIFFERENTIAL

Pattern	Distinguishing Feature
COMMON CAUSES	
Cold hands AND feet	Spleen- or Stomach-Yang deficiency
Cold hands ONLY	Lung- and/or Heart-Yang deficiency
Cold feet ONLY	Kidney-Yang deficiency
Cold hands and feet in WOMEN	Blood deficiency
Cold hands in women specifically	Heart-Blood deficiency
Cold feet in women specifically	Liver-Blood deficiency
Cold FINGERS AND TOES specifically	Liver-Qi stagnation (Qi can't reach extremities)

LESS COMMON CAUSES

Cold hands/feet + contradictory hot symptoms	Phlegm or interior Heat obstructing Qi circulation
Cold hands + intense Heat signs elsewhere	Heat in Pericardium at Nutritive Qi level (Four Levels) — Heat trapped, obstructing circulation to extremities

DORSUM VS PALM — FULL-HEAT VS EMPTY-HEAT

Hot DORSUM (back) of hand	Full-Heat
Hot PALM of hand	Empty-Heat

The 發熱 fā rè distinction: Classical "fever" in exterior Wind invasion actually refers to the objective hot feeling of the dorsum of hands/forehead on palpation — NOT necessarily a measured fever. Spiritual Axis Ch.74: hot palm = Heat in Intestines; cold palm = Cold in Intestines.

PALPATION OF HANDS & FEET — TEMPERATURE DIAGNOSIS

Lung- and/or Heart-Yang deficiency	Coldness of hands ONLY
Spleen- and/or Kidney-Yang deficiency	Coldness particularly of feet, but often hands too — WHOLE LIMBS cold
Stomach-Yang deficiency	Coldness of both hands and feet
Heart-Blood deficiency (women)	Cold hands
Liver-Blood deficiency (women)	Cold feet
Liver-Qi stagnation	Cold hands/feet, particularly fingers and toes — NOT a Yang deficiency, it's stagnant Qi unable to reach extremities

Less common causes: Phlegm in the Interior obstructing Qi circulation to limbs (may also occur with Phlegm-Heat, producing contradictory hot/cold symptoms) · Very pronounced interior Heat obstructing Qi circulation so it cannot warm hands/feet — produces contradictory hot/cold symptoms (e.g., Heat in Pericardium at Nutritive Qi level: Dark-Red tongue no coating, night fever, mental restlessness — *but cold hands*).

PALPATING THE HAND — ORGAN CORRESPONDENCE MAP

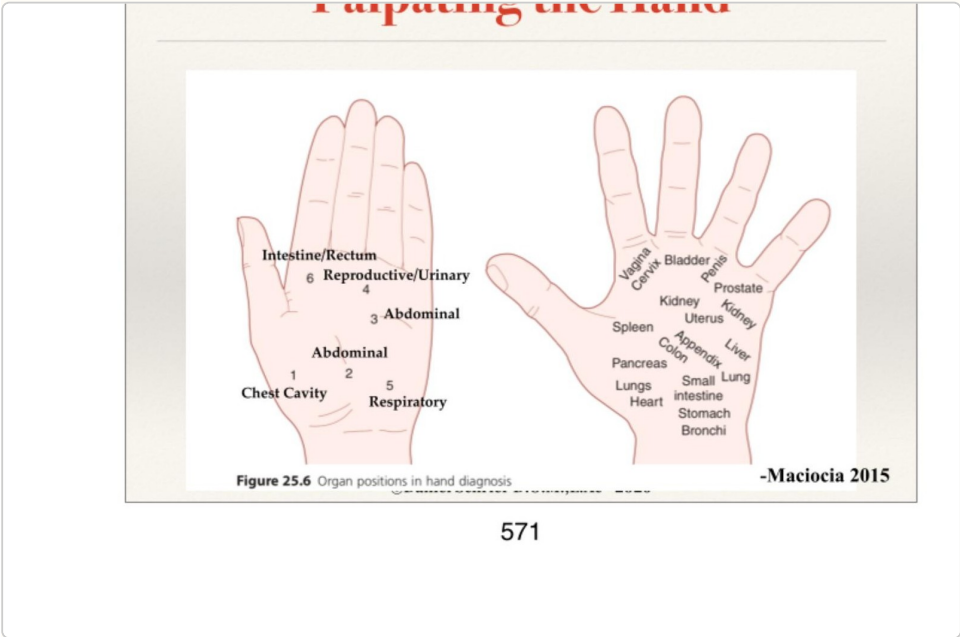


Figure 25.6 — Organ positions in hand diagnosis (Maciocia 2015)

The palm reflects internal organs in a pattern similar to the ear — visualize a small baby superimposed on the palm. Diagnosis: gently press the relevant area. **Sharp pain** = Full condition. **Dull soreness** = Empty condition of that organ.

Hand Zone	Corresponds To
Chest cavity organs	Lungs, Heart, Bronchi
Upper abdominal cavity	Liver, Spleen, Stomach, Pancreas
Lower abdominal cavity	Small intestine, Colon, Appendix
Reproductive/urinary organs	Kidney, Bladder, Uterus, Prostate, Cervix, Penis, Vagina
Respiratory organs	Lungs, Bronchi
Intestines/rectum area	Colon, Rectum

APICAL BEAT — "INTERIOR EMPTINESS" (XU LI 虚里)

- ❖ Refers to the pulse of the heart of the left ventricle, which can be felt in the fifth intercostal space
- ❖ Described in Su Wen 18
- ❖ Clearly felt, not hard, relatively slow: Normal
- ❖ Feeble and without strength: deficiency of the Gathering Qi Zong Qi
- ❖ Strong and hard: excess condition of the Lungs and/or Heart
- ❖ Large but empty: Heart-Qi deficiency
- ❖ Stopping and starting: severe shock or alcoholism
- ❖ Discrepancy between apical and radial pulse: poor prognosis
- ❖ Rapid: shock, fright or outburst of anger

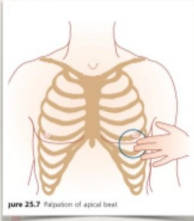


Figure 25.7 Palpation of apical beat (Maciocia 2015)

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Figure 25.7 — Palpation of apical beat (Maciocia 2015)

Palpate over the left ventricle apex. Traditionally considered the end of the Stomach Great Connective Channel and reflective of the Gathering Qi (Zong Qi) of the chest.

Finding	Meaning
Clear, not hard, relatively slow	Normal
Feeble, without strength	Deficiency of Gathering Qi (Zong Qi)
Strong and hard	Excess condition of Lungs and/or Heart
Large but empty	Heart-Qi deficiency
Stopping and starting	Severe shock or alcoholism
Discrepancy between apical and radial pulse	Poor prognosis
Rapid	Shock, fright, or outburst of anger
Pulsation cannot be felt at all	Phlegm or hiatus hernia

AREA BELOW THE XIPHOID PROCESS

- ❖ Reflects the state of the Upper Burner Qi (Lung and Heart Qi and Zong Qi/ Gathering Qi)
- ❖ Should feel soft relative to the rest of the abdomen
- ❖ Hard on palpation: Full condition, Qi stagnation (from emotional problems)
- ❖ Too soft on palpation: Heart deficiency

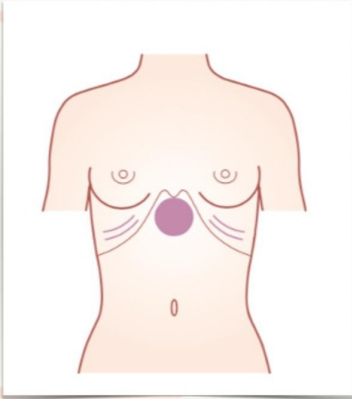


Figure 25.8 Area below the xiphoid process (Maciocia 2015)

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Figure 25.8 — Area below the xiphoid process (Maciocia 2015)

Should feel relatively SOFTER than the rest of the abdomen — this reflects Upper Burner Qi (Lung, Heart, Gathering Qi). Compare directly to surrounding abdomen.

Relatively soft (as expected)	Normal — smooth flow of Lung/Heart-Qi
Hard on palpation	Full condition, Qi stagnation from emotional problems — constriction of Corporeal Soul
Too soft on palpation	Heart deficiency

PALPATING THE CHEST — CENTER VS SURROUNDING

Very tender, light palpation, center (Ren-17 Shanzhong)	Heart-Blood stasis
Tender around the center	Excess condition of Lungs — often Phlegm retention
Palpation RELIEVES discomfort	Deficiency condition of Heart or Lungs
Light palpation relieves, deep pressure elicits discomfort	Combined Deficiency and Excess

PALPATING THE BREAST (LUMPS)

Critical caveat: Chinese medicine breast palpation is NEVER a substitute for Western diagnosis. Never rely on TCM palpation to distinguish benign from malignant — its purpose is identifying the CM pattern only.

Quality	Indicates
Relatively soft	Phlegm
Relatively hard	Blood stasis
Distinct edges	Phlegm
Indistinct edges	Toxic Heat
Mobile on palpation	Phlegm
Immovable on palpation	Blood stasis or Toxic Heat

Clinical Picture	Likely Diagnosis (CM pattern)
Small, movable, distinct edges, size varies w/ cycle	Fibrocystic disease — Phlegm + Qi stagnation
Single, hard, movable, distinct edges, mild pain	Fibroadenoma — Phlegm + Blood stasis
Single, hard, IMMOVABLE, indistinct margins, painless	⚠ Possible carcinoma — Phlegm + Qi stag + Blood stasis, Penetrating/Directing Vessel disharmony

ABDOMINAL PALPATION — QUICK REFERENCE

❖ Discussed in the *Nan Jing* and *Shang Han Lun*

❖ Practitioners palpate various areas on the abdomen

❖ By discerning the sensitivity and palpations of these locations they then make inferences about the balance of the various Organs

❖ Japanese physicians would further evolution and methods of diagnosis.



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Abdominal palpation technique (Schrier Lecture Slides)

Finding	Indicates
Solid, resilient, elastic	Good Original Qi — NORMAL
Hard / painful on palpation	Full condition
Too soft / ache relieved by palpation	Empty condition
Masses that MOVE under fingers	Qi stagnation
Masses FIXED and hard	Blood stasis

Core palpation principle (applies everywhere on the body): Moving mass = Qi. Fixed mass = Blood. This single rule underlies chest, abdomen, and channel-point palpation alike.

DIAGNOSTIC POINT CATEGORIES 診斷穴 — OVERVIEW

Point Category	Function
Back-Shu (Transporting) Points	Where Qi/Blood of a specific organ "infuses" on the back — full table on next page
Front-Mu (Collecting) Points	Particularly reactive to pathological internal-organ changes — full table on next page
Lower He-Sea Points	Used for diagnosis of the 6 Fu (Yang) organs specifically — full table on next page
Ah Shi Points	"Oh yes!" points — tender spots with no fixed location, found by palpation alone
Sharp pain (spontaneous or on pressure)	Full condition of the relevant organ
Dull soreness (spontaneous or on pressure)	Empty condition of the relevant organ

NAN JING 16/56 — FIVE-PHASE ABDOMINAL PALPATION MAP

- Below the sternum reflects
 - Heart/Fire (Ren 12-15)
- Around the umbilicus reflects
 - Spleen/Earth (Ren 7-Ren 12)
- Below the umbilicus reflects the
 - Kidney/Water (Ren 7-Pubis)
- Right side below the rib cage reflects
 - Lung/Metal (Right Abdominal Region)
- Left side below the rib cage reflects
 - Liver/Wood (Left Abdominal Region)

Matsumoto K, Birch, S (1988) Hara Diagnosis: Reflections of the Sea
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5-Phase Abdominal Palpation Map (Matsumoto & Birch 1988, Hara Diagnosis: Reflections of the Sea)

Below the sternum	Heart / Fire (Ren-12 to Ren-15)
Around the umbilicus	Spleen / Earth (Ren-7 to Ren-12)
Below the umbilicus	Kidney / Water (Ren-7 to Pubis)
Right side below rib cage	Lung / Metal (Right Abdominal Region)
Left side below rib cage	Liver / Wood (Left Abdominal Region)

JAPANESE ABDOMINAL DIAGNOSIS — FUKU SHIN 腹診

- Waichi Sugiyama (1610-1694)
 - Credited for the development of acupuncture as we know today in Japan and the further contributions to abdominal diagnosis
- Todo Yoshimasu (1702-1773)
 - Would develop the practice by creating diagnostic indicators gently palpating the abdomen and associating it with a traditional herbal formula
- Bunrei Inaba & Yoshitora (Shukuko) Wakuda
 - further the development of Fuku Shin in the 19th Century

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Fuku Shin abdominal zone map (Schrier Lecture Slides)

Waichi Sugiyama (1610–1694)	Credited for developing acupuncture as practiced in Japan today and contributing to abdominal diagnosis
Todo Yoshimasu (1702–1773)	Developed the practice by gently palpating the abdomen and associating findings with a traditional herbal formula
Bunrei Inaba & Yoshitora (Shukuko) Wakuda	Further developed Fuku Shin in the 19th century

MASTER TUNG PALM DIAGNOSIS 掌診

Master Tung Palm Diagnosis

- ❖ Examine the palm for blood vessels, abnormal spots tenderness etc.
- ❖ From index finger to LU 10 = Lung
- ❖ From middle finger to PC 8 = Heart
- ❖ From ring finger to wrist = Liver & Spleen
- ❖ From pinky to wrist = Kidneys



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Master Tung Palm Diagnosis — annotated hand (Schrier Lecture Slides)

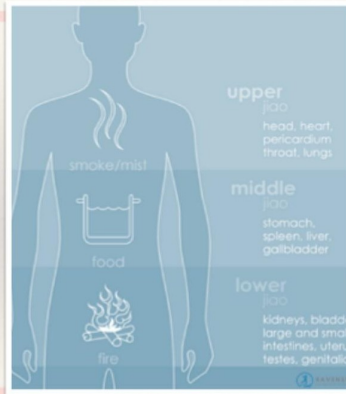
Index finger to LU-10	Lung zone
Middle finger to PC-8	Heart zone
Ring finger to wrist	Liver & Spleen zone
Pinky to wrist	Kidneys zone

Vessel Color	Meaning	Skin Color	Meaning
Light blue	Cold and deficiency	Blue or black	Pain
Dark blue	Pain and excess	Red or yellow	Heat
Red or purple	Heat and inflammation	White	Cold
—	—	Yellow and atrophy	Deficiency

Very dark/gloomy color = pathogen has invaded to interior. Light color = pathogen still on exterior. Palpate palm: soft+atrophy=deficiency; firm+tender=excess; warm=warm abdomen; cold=cold abdomen.

PALPATING THE THREE JIAO 三焦

- ❖ Upper Jiao situated in the chest and lies above the diaphragm.
- ❖ Contains the Heart, Pericardium and Lung
- ❖ Middle Jiao lies between the diaphragm and the navel.
- ❖ Contains the Stomach, Spleen, Liver and Gall Bladder, and joins the Lower Burner at the navel
- ❖ Lower Jiao lies below the navel.
- ❖ Contains the Small and Large Intestines, Bladder and Kidney.



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Upper / Middle / Lower Jiao — organ contents by region (Schrier Lecture Slides)

Upper Jiao	Chest, above diaphragm — Heart, Pericardium, Lung
Middle Jiao	Between diaphragm and navel — Stomach, Spleen, Liver, Gall Bladder; joins Lower Burner at navel
Lower Jiao	Below navel — Small/Large Intestines, Bladder, Kidney

Purpose of Three Jiao palpation: assess warmth/strength of Qi in each Burner · determine if moxibustion/warming should be significant in treatment · assess treatment progress over time · assess patient's reaction to physical contact.

WHAT PALPATION CAN TELL US

Information about various areas of the body, the meridians, and acu-points
Overall condition & general state of meridians and organs

Finding Category

- Stagnations (local or generalized)
- Ah Shi / Trigger Points
- Channel involvement
- Severity of condition
- Organs and ligaments (health, flexibility, location)
- Adhesions/obstructions (stagnations, blockages, dampness)

CHANNEL PALPATION IN THE CLASSICS

Involves tracing the pathways of the 12 primary channels to determine quality of Qi and Blood flow. Discussed in Huangdi Neijing (Su Wen 20, Ling Shu 20/51/73/75), Nan Jing 13, Zhang Zhongjing's Shang Han Lun, Sun Simiao's Qian Jin Yao Fang, and Li Shizhen's Qijing Ba Mai Kao.

3 TECHNIQUES IN PALPATING THE CHANNELS

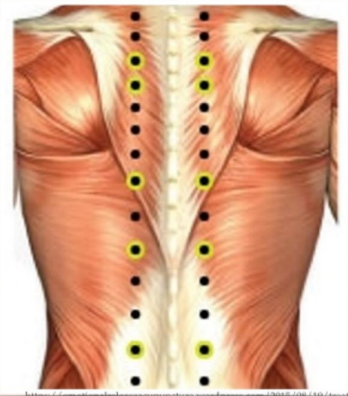
Technique	Method & Interpretation
① Palpation	Running fingers along the course of the channels, often below elbows/knees. Palpate 3 times, pressing a bit harder each pass — some changes are clear at the surface but not deeper, and vice versa. Notice structural changes (muscle tension, nodules, bumpiness).
② Pressing	Use thumb/fingertips to check tenderness along a channel or at specific points. Tenderness/pain = Excess condition. More pain = accumulation of Xie Qi (pathogenic Qi). Weak & soft with/without tenderness = Qi stagnation. Slippery and/or hard = Phlegm.
③ Feeling	Discern noticeable changes in temperature, moisture, skin texture. Compare areas of the body to each other (not necessarily restricted to the channels).

AGGRESSIVE ENERGY (AE) TREATMENT — 5-ELEMENT CONTEXT

Aggressive Energy (AE) Treatment

- ❖ 5 Element Worsley treatment block created by “toxic” qi circulating along the Ke Cycle/Control Cycle between the yin organs
- ❖ Hinted at in *Su Wen* Chapter 65
- ❖ Usually comes from an internal cause ie.. family/relationship problems, financial worries, work issues, or shocks. Though it can come from external causes
- ❖ Points used: BL 13, BL 14, BL 15, BL 18, BL 20, BL 23

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<https://emotionalreleaseacupuncture.wordpress.com/2016/08/19/treatment-5-aggressive-energy-calming-the-organs/>

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AE treatment points on the back (Schrier Lecture Slides)

5-Element Worsley treatment block created by "toxic" Qi circulating along the Ke (Control) Cycle between the Yin organs
Hinted at in *Su Wen* Chapter 65
Usually comes from an internal cause — family/relationship problems, financial worries, work issues, or shocks. Can also come from external causes.
Points used: BL-13, BL-14, BL-15, BL-18, BL-20, BL-23

Tables

Universal rule for all three tables below: Sharp pain (spontaneous or on pressure) = Full condition of that organ. Dull soreness (spontaneous or on pressure) = Empty condition of that organ.

BACK-SHU (BACK-TRANSPORTING) POINTS 背俞穴 — BLADDER CHANNEL

Back Shù俞

- Shù 俞 could be translated as “transport” indicating the qi of the 12 Channels is transported to the surface and is accessible through these points.
- Located on the back the shù 俞 points access particularly strong reserves of qi to provide strong tonification or sedation to the function of their corresponding organ.
- Note: that the shù 俞 points are located relatively over their respective organs

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12 Points

BL 14 (PC)
BL 15 (HT)
BL 18 (LR)
BL 19 (GB)
BL 20 (SP)
BL 21 (ST)
BL 22 (TE)
BL 23 (KI)
BL 25 (LI)
BL 27 (SI)
BL 28 (BL)

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12 Back-Shu points labeled on the back (Schrier Lecture Slides)

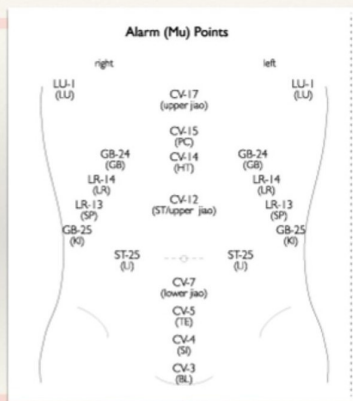
Located on the first lateral line of the back, 1.5 cun lateral to the spine (except BL-14/15 pericardium/heart pairing nuance). Each point is where that organ's Qi and Blood "infuses" — the most direct back-diagnostic point for its organ.

Point	Name	Organ	Vertebral Level
BL-13	Feishu	Lung	T3, 1.5 cun lateral
BL-14	Jueyinshu	Pericardium	T4, 1.5 cun lateral
BL-15	Xinshu	Heart	T5, 1.5 cun lateral
BL-18	Ganshu	Liver	T9, 1.5 cun lateral
BL-19	Danshu	Gallbladder	T10, 1.5 cun lateral
BL-20	Pishu	Spleen	T11, 1.5 cun lateral
BL-21	Weishu	Stomach	T12, 1.5 cun lateral
BL-22	Sanjiaoshu	Triple Burner	L1, 1.5 cun lateral
BL-23	Shenshu	Kidney	L2, 1.5 cun lateral
BL-25	Dachangshu	Large Intestine	L4, 1.5 cun lateral
BL-27	Xiaochangshu	Small Intestine	S1, 1.5 cun lateral
BL-28	Pangguangshu	Bladder	S2, 1.5 cun lateral

ADDITIONAL DIAGNOSTIC BACK POINTS (BEYOND STANDARD BACK-SHU SET)		
BL-43 Gaohuangshu	Reflects the state of the Lungs — often used when LU-Shu (BL-13) is not sufficiently reactive	
BL-53 Zhishi	Often sore in Kidney disease — a secondary Kidney diagnostic point	
BL-31 Shangliao	These four (the "Eight Liao" upper points) reflect the state of the reproductive system, particularly in women	
BL-32 Ciliao		
BL-33 Zhongliao		
BL-34 Xialiao		

FRONT-MU (ALARM / COLLECTING) POINTS 募穴

- ❖ Are also called “Alarm Points” which signifies that the points tend to become sensitive to pressure as qi collects in the channel
- ❖ Are the places where qi of those organs “collect” or “accumulate”.
- ❖ Located on the front surface of the body, providing deep access to the yin aspect of each official's realm of functioning



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Alarm (Mu) Points — anterior torso locations, both sides (Schrier Lecture Slides)

Located on the front/chest/abdomen, on the channel most anatomically close to the organ — NOT necessarily on that organ's own channel. Particularly reactive to acute or Full conditions of their organ.

Point	Name	Organ	Location
LU-1	Zhongfu	Lung	Lateral chest, 1st intercostal space level, 6 cun lateral to midline
REN-17	Shanzhong	Pericardium	Midline, level with 4th intercostal space (nipple line)
REN-14	Juque	Heart	Midline, 6 cun above umbilicus
LIV-14	Qimen	Liver	6th intercostal space, directly below nipple
GB-24	Riyue	Gallbladder	7th intercostal space, below LIV-14
LIV-13	Zhangmen	Spleen	End of 11th rib (free-floating rib tip)
REN-12	Zhongwan	Stomach	Midline, 4 cun above umbilicus
REN-5	Shimen	Triple Burner	Midline, 2 cun below umbilicus
GB-25	Jingmen	Kidney	Free end of 12th rib (lateral)
REN-4	Guanyuan	Small Intestine	Midline, 3 cun below umbilicus
REN-3	Zhongji	Bladder	Midline, 4 cun below umbilicus
ST-25	Tianshu	Large Intestine	2 cun lateral to umbilicus

LOWER HE-SEA POINTS 下合穴 — SIX FU ORGANS ONLY

Unique to the six Fu (Yang) organs — three sit directly on their own channel (ST/GB/BL), three “borrow” a Lower He-Sea point on the Stomach or Bladder channel because their own channel doesn’t reach the lower leg with a suitable He-Sea point.

Point	Name	Fu Organ	Note
ST-36	Zusanli	Stomach	On its own channel
GB-34	Yanglingquan	Gallbladder	On its own channel
BL-40	Weizhong	Bladder	On its own channel
ST-37	Shangjuxu	Large Intestine	“Borrowed” — on Stomach channel
ST-39	Xiajuxu	Small Intestine	“Borrowed” — on Stomach channel
BL-39	Weiyang	Triple Burner	“Borrowed” — on Bladder channel

AH SHI POINTS 阿是穴 — “OH YES!” POINTS

No fixed location, no fixed name — identified purely by palpation finding a tender or reactive spot. Named for the patient's exclamation (“Ah, yes — right there”) when the point is pressed. Common in musculoskeletal/Bi Syndrome presentations, and can appear anywhere along a channel or off-channel entirely.

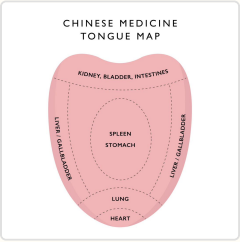
PATTERN NAME	DISEASE CATEGORY	WEEK / QUIZ

ETIOLOGY	ORGAN SYSTEM	DATE

CLINICAL ESSENTIALS — INQUIRY + PALPATION FINDINGS

INQUIRY 問診 · KEY FINDINGS

CHINESE MEDICINE TONGUE MAP



PAIN / THIRST / SWEAT / SLEEP / CAUSE

PALPATION 切診 · SKIN / CHEST / ABDOMEN / POINTS

TEMP / TEXTURE / MASSES / POINT TENDERNESS

PULSE 脈診 · CUN · GUAN · CHI

Wrist	CUN 寸	GUAN 關	CHI 尺
L HT/SI		LV/GB	KD/BL
R LU/LI		SP/ST	PC/TB

QUALITY / OVERALL

TREATMENT PRINCIPLE 治則

KEY POINTS / HERBS

[illegible]

[illegible]

[illegible]

